



GUIDELINE

Managing Clinical Risk After Disclosure of Child Sexual Abuse

Scope (Staff):	All CAMHS clinical staff
Scope (Area):	All CAMHS services

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with:
[this disclaimer](#)

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Aim

To guide Child and Adolescent Mental Health Services (CAMHS) clinical staff in the optimal management of risk of suicide and/or self-harm when a disclosure of child sexual abuse (CSA) is made. Some parts of this guideline may also be relevant for managing other aspects of clinical risk that could arise as a result of child abuse e.g. violence and aggression, sexualised behaviours, drug and alcohol use, although these risk factors are not the focus of this document.

Risk

Inadequate assessment and support of children following the disclosure of sexual abuse may increase the risk of suicide, self-harm, further psychological harm, and potentially, further abuse.

Background

Children who disclose CSA to a trusted adult and receive support show greater psychological adjustment and fewer externalising behaviours and symptoms of post-traumatic stress. They are also less likely to experience further abuse.^{1,2} The period following the disclosure or discovery of CSA may, however, be a period of increased vulnerability for some children. Increased risk of suicide, self-harm, and adjustment problems may occur.^{3,4,5}

The following key factors may affect a child's level of risk following disclosure and should be taken into consideration by the treating team:

- the presence of complex trauma (for example, as a result of domestic violence, parental substance use, emotional abuse / neglect, or physical abuse)
- time elapsed between the abuse incident(s) and disclosure
- the child and family's relationship to the alleged perpetrator ⁶
- the reactions of significant people in the child's life to the disclosure⁷
- parental/caregiver support⁸
- gender
- age at the time the abuse occurred
- system responses
- the child's perceptions of the impact of the disclosure.

Ensuring that children and families who have experienced abuse feel supported is a vital part of safety throughout the disclosure process.

Definitions

Child: An infant, child, adolescent or young person under the age of 18 years.

Child Abuse: All forms of physical and/or emotional ill-treatment, sexual abuse, neglect or negligent treatment or commercial or other exploitation, resulting in actual or potential harm to the child's health, survival, development or dignity in the context of a relationship of responsibility, trust or power.

Child Sexual Abuse: In relation to a child includes sexual behaviour in circumstances where a child is exposed to, or involved in, sexual activity that is inappropriate to the child's age and developmental level. It includes circumstances where:

- The child is the subject of bribery, coercion, a threat, exploitation or violence; or
- The child has less power than another person involved in the behaviour; or
- There is a significant disparity in the developmental function or maturity of the child and another person involved in the behaviour

Some examples of child sexual abuse include (but are not limited to):

- Letting a child watch or read pornography
- Allowing a child to watch sexual acts
- Fondling the child's genitals
- Having oral sex with a child
- Vaginal or anal penetration
- Using the internet to find a child for sexual exploitation

Forming a belief: Means a Mandatory Reporter is more inclined to accept than reject their belief that child sexual abuse has occurred.

Reasonable grounds: Means most people would come to the same or somewhat similar conclusion based on the information provided. Professional judgement and objective observation which identifies warning signs or possible indicators of child sexual abuse and contributes to 'reasonable grounds'. Knowledge of child development and consultation with colleagues or other professionals can also contribute to 'reasonable grounds'.

Scope

CAMHS' role is to support the child and family following a disclosure of CSA. Mental health assessment and treatment undertaken by CAMHS clinicians is separate to any formal investigations of the abuse undertaken by Police and the Department of Communities following an allegation of CSA. Where a CAMHS clinician has concerns around influencing an investigative / legal process they should:

- Consider contacting the Perth Children's Hospital (PCH) [Child Protection Unit \(CPU\)](#) for guidance
- Consider contacting [CAHS Mediation and Legal Support Services](#) for guidance
- Consider consulting the [Guidelines for Protecting Children 2020](#)
- Ensure that they only ask open-ended questions and record all answers verbatim.

Principles

The best interests of the child are paramount. Children who have experienced abuse, and their families, may be particularly vulnerable and encounter unique challenges in accessing mental health services. Services must be sensitive to the child and family's needs, underpinned by the principles of trauma-informed practice, and committed to enhancing the child's physical and emotional safety.

This procedure is underpinned by both the Charter of Mental Health Care Principles ([Mental Health Act 2014](#)) and the [Child Safe Standards](#) developed by the Royal Commission into Institutional Responses to Child Sexual Abuse. See [Appendix 1: Statement of applicability against the Charter of Mental Health Care Principles in the Mental Health Act 2014 and Child Safe Standards Developed by the Royal Commission into Institutional Responses to Child Sexual Abuse](#), which describes how each Principle and Standard relates to this Procedure.

This guideline also acknowledges the possible effects that disclosure of CSA may have on CAMHS staff, and provides details regarding staff support, including links to the CAHS Employee Assistance Program (EAP) and relevant training opportunities.

Procedure

For information on who to contact and the responsibilities of those services / agencies see [Appendix 2: Responsibilities \(who to contact\)](#).

For a summary of the procedure see [Appendix 3: Flowchart](#).

Remember: Only ask open-ended questions. Do not ask leading/direct questions as this may prejudice any subsequent investigation by relevant officers.

Steps	Additional Information
1. Acknowledge the disclosure a) Provide validation by expressing belief and trust in what the child said happened in relation to the abuse. b) Acknowledge their courage in disclosing/how difficult it must be to discuss the abuse. c) Discuss duty of care with the child and their family, focusing on the need to ensure their safety. Wherever possible, the clinician must be open and clear with the child and their family about the steps the clinician will take following the disclosure (e.g. who will be contacted, when, and why). i. On some occasions a clinician may determine that it is not safe for the child or others (including CAMHS staff) to immediately inform the child or parent/guardian that a report is being made. On these occasions a plan should be established to determine when and how to inform the child or their parent/guardian that a report is being made. ii. Note it is an offence under the mandatory reporting of CSA obligations contained in the Children and Community Services Act 2004 (CCSA) to disclose the identity of the person who has made a mandatory report of CSA, with certain exceptions (see s.124F of CCSA).	If a Mandatory Reporter (doctors, nurses or midwives) on reasonable grounds forms a belief that sexual abuse is occurring to a child (under 18 years of age), then this must be reported in writing to the Department of Communities Mandatory Reporting Service. Refer to the CAHS Mandatory Reporting of Child Sexual Abuse for more information. If non-mandated staff recognize and form a belief that child sexual abuse has or is occurring, they should formally report their concerns to the Department of Communities using the online Child Protection Concern Referral Form and should refer to CAHS Child Safety and Protection for more information. Note that in all instances where any staff member identifies concerns about any form of child abuse (i.e. has recognized that a child may have been harmed or is at current risk of harm through child abuse), they must formally report their concerns to the Department of Communities using the online Child Protection Concern Referral Form. More information can be found in both the CAHS policies referenced above.

Steps	Additional Information
iii. The extent to which the child is included in these discussions will depend on a range of factors, including (but not limited to) the child's age, maturity, mental state, and ability to understand the discussion. iv. If possible, maximise the comfort of the young person during the disclosure, e.g. consider their preferences of gender or cultural background of the person to whom they are disclosing.	
2. Take initial steps to assess and enhance short-term physical and psychological safety a) From a child protection perspective, clinicians must: i. Assess the child's immediate safety from the alleged perpetrator, and with their caregiver. Assess the immediate safety of other children in contact with the alleged perpetrator ii. Notify the Department of Communities in accord with the Guidelines for Protecting Children (see pg. 49 – Reporting Child Abuse) and the CAHS Child Safety and Protection policy. It is the Department of Communities responsibility to assess ongoing safety of the child iii. Contact the relevant service (i.e. CPU or SARC - see right) to determine whether there is a need for medical and / or forensic assessment. - If a referral is made, notify the family. Discuss with CPU / SARC to clarify processes and responsibilities around obtaining consent for assessment.	Unless immediate or urgent assessment is required, medical / forensic assessment is normally discussed at the joint strategy meeting by WA Police and Department of Communities. However, CAMHS staff should refer to the CAHS Child Safety and Protection for more information. • The PCH Child Protection Unit (CPU) provides medical / forensic assessment for children under 16 years of age; • Sexual Assault Resource Centre (SARC) provides medical / forensic assessment for children over 16 years of age, or children between 13 and 16 years of age who have been abused by someone <u>outside their family</u>.

Steps	Additional Information
b) From a mental health perspective, clinicians must: i. Assess for risk of suicide and self-harm in accordance with the CAMHS Risk Assessment and Management Policy ii. Consult with the internal Multi-disciplinary Team (MDT) iii. Where relevant, liaise with the external team (e.g. WA Police Child Abuse Squad (CAS), School, Department of Communities, or other Agency involved with the Child). c) Where the child is assessed as being at heightened risk of suicide or self-harm: i. Follow procedures outlined in the CAMHS Risk Assessment and Management Policy ii. Consult with the Head of Service/ Service Manager/ Senior Clinician to determine the most appropriate care setting for the child. - If inpatient care is required, consult with the CAMHS Emergency Telehealth Service (ETS) on 1800 048 636.	Refer to Section 3 of the CAMHS Clinical Handbook (Risk Assessment). Refer to the CAMHS Shared Care Guidelines and CAMHS MDT Review Policy. If there is difficulty in accessing an inpatient unit admission or differences exist in assessment of clinical risk, this can be escalated for clinical review, as per the CAMHS Clinical Assessment Policy and CAMHS MDT Review Policy. If required, further escalation can be made to the CAMHS Executive via the Directorate Head of Department. CAMHS acknowledges that there is a cohort of children who need to stay in hospital in order to feel physically and emotionally safe in the period following disclosure of sexual abuse. Inpatient admission may provide an appropriate containment and stabilisation of acute distress.
3. Undertake clinical assessment of all mental health risks in the context of the disclosure a) Contextual issues that must be considered as part of the assessment include: i. Whether the abuse is a once-off incident or occurred over a period of time ii. What developmental stage(s) the child was in at the time of the abuse	The focus of the clinical assessment is on assessment of risk and identifying the child's current support needs, and not about obtaining details about the alleged abuse. Clinicians should avoid asking the child to repeat details of the abuse that he/she has given to other agencies such as police or child protection. Where possible, some of this information can be sought from Department of Communities or Police.

Steps	Additional Information
iii. What kind of relationship the child has with the alleged perpetrator iv. What kind of relationship the parent(s)/ family has with the alleged perpetrator, and how this might affect their response to the disclosure v. the implication(s) of the disclosure for the child (e.g., whether they feel exposed/ vulnerable, guilty, afraid, confused/ conflicted) vi. the child's fears around the disclosure (e.g. response from their family, separation from others or the perpetrator, possible conviction/ imprisonment of the alleged perpetrator) vii. Where relevant, the parents'/family's fears around the disclosure. b) Open-ended questions must be used when asking the child about the abuse (e.g. "tell me more about that"), and the emotional safety and welfare of the child should be the primary focus. The clinician must avoid probing the child about details of the abuse incident(s), to avoid leading questions or lines of enquiry, or causing the child to disclose more than they presently feel safe to share. c) Assess the child and family's level of engagement and manage risk of non-attendance/disengagement following disclosure as per the procedure described in the CAMHS Clinical Engagement Policy. In some cases, despite the best efforts of staff to maintain therapeutic relationships, the child and family may disengage from CAMHS due to a loss of trust resulting from a mandatory report of CSA. Disengagement from services after disclosure of abuse precludes ongoing,	

Steps	Additional Information
active monitoring of risk and, in some circumstances, may indicate an increase in the child's risk status. In the event of disengagement, the CAMHS team must advise the relevant lead agency (e.g. CPU, WA Police, SARC, Department of Communities) of any concerns regarding the child's mental health and wellbeing. This must be documented in the medical record.	
4. Determine intervention/support needs a) It is important that clinicians are aware of the need to provide support to the child and family throughout and following potentially long investigative and legal processes. The longer-term support needs of the child and their family must be considered (in consultation with the MDT). Support must be provided for as long as is clinically indicated and may be provided by CAMHS or other agencies (e.g. CPU, SARC). b) Clinicians must seek input from their MDT. The MDT, together with the clinician, considers whether the following are indicated: - Co-management of the case with another service or agency (e.g. schools, Department of Communities, CPU) - Consultation with another service or agency - Involvement of the relevant CAMHS Child Protection Consultation Liaison (CPCL) workers in the case - Referral to Touchstone (Note: Touchstone is a day program and does not provide emergency / urgent treatment. The service may be appropriate for longer-term management of clinical risk).	The following resources may assist CAMHS clinicians in determining appropriate intervention / support: • CAMHS Shared Care Guidelines • NICE Guideline – Child Abuse and Neglect (see section 1.7 - <i>Therapeutic interventions for children, young people and families after child abuse and neglect</i>) • Good practice in providing services: Victims of child abuse. The child will be welcome to continue with CAMHS while the existing presenting mental health concerns are being addressed. After this, it may be appropriate to discuss discharge with the family. Clinicians should advise the child and family that the child may be re-referred to CAMHS at any time and explore options for ongoing support if required.

Steps	Additional Information
c) Consider whether there is a need to initiate a Shared Care Agreement with Department of Communities and other services, such as CPU, engaged with the child and family. d) Consider the child and family's support needs in the period following return to their normal home environment. e) Consider the child and family's cultural and spiritual needs, including: i. whether traditional Aboriginal healing approaches may be a useful component of therapeutic treatment. If so, it is important to consider: - the family's preferences in relation to the type of treatment and treatment provider - any additional supports the child may require to access traditional healing (e.g. if the child's carer / guardian is not an Aboriginal person they may be unfamiliar with traditional healing approaches) - at what stage in a child's episode of care it is clinically indicated / in the client's best interest for a Traditional Healer to be involved (for example, if acutely psychotic, it may be indicated to wait until symptoms are less acute). A psychiatrist should be involved (working collaboratively with the Aboriginal Mental Health Worker) in determining this. ii. the child and family's preferences regarding the gender of the CAMHS clinician.	The Shared Care Agreement should clearly articulate the role of each agency in supporting the child and family, including which aspects of the child's mental health are to be addressed by the CAMHS team. The Shared Care Agreement form is included as an appendix in the CAMHS Shared Care Guidelines and may be used for this purpose.

Steps	Additional Information
f) Continuity of care, including consistency and regularity of support, must be addressed and articulated within the management plan for the child and family. The clinician's role is to: i. Establish a trusting adult relationship between the therapist and child, providing the foundation for a corrective attachment experience where needed. The establishment of trust and rapport with a child who has disclosed sexual abuse may be a prolonged process, and it is important to dedicate sufficient time (which may include longer or additional appointments) to this process. ii. Strengthen the family capacity for care, protection and sustainable emotional support, including extending the corrective attachment experience between the child and therapist, to the primary caregiver. iii. Share relevant and appropriate information with Department of Communities g) Vicarious trauma is an occupational hazard and a health and safety issue for CAMHS clinicians. When working with children and families who have experienced abuse, clinicians should prioritise their own support needs and contact their Service Manager to discuss what support they require (e.g. debriefing sessions, time out, peer support / supervision, time off work, CAHS Employee Assistance Program).	Refer to: • CAMHS Clinical Supervision Policy • CAHS Critical Incident Impact Management (Debrief) Policy • CAHS Employee Assistance Program (EAP)

Steps	Additional Information
5. Advocate for the child through Interagency work a) CAMHS clinicians may need to adopt an advocacy role in promoting the mental health needs of a child who has disclosed abuse with Department of Communities and other agencies. This includes advocating for acknowledgment that CAMHS clients may be at increased risk and require special consideration when an allegation of CSA is investigated. b) CAMHS clinicians may need to escalate the case to the relevant Department of Communities Team Leader and/or District Director if their concerns for the child are not fully considered. c) If the concerns are not addressed through this avenue, clinicians must seek support from their own management structure in further advocating for the child. This may involve escalation of the case, through the Service Manager and Head of Service, to the CAMHS Executive.	Refer to CAMHS Shared Care Guidelines and Agreement form. The clinician, through mandatory and non-mandatory reports of child sexual abuse, should use the Mandatory Report of CSA Form to highlight any special considerations (e.g. likelihood of elevated mental health risk) that Department of Communities should take into account in assessing and managing the case. They may indicate on the referral that consideration be given to them being involved in the Department of Communities/WA Police Joint Strategy Meeting relating to the report.
6. Report and document a) Document the disclosure in the child's medical record as soon as possible after the session including, wherever possible, the exact: i. question asked by the clinician immediately prior to the disclosure ii. words used by the child and/or other person making the disclosure. b) Keep accurate clinical notes that include all consultations and referrals. c) Update the Child's Care Plan, documenting CAMHS' response to the disclosure.	Refer to State Records Office - Guidance for identifying and retaining records which may become relevant to an actual or alleged incident of child sexual abuse for information on the creation of records as part of a response to an allegation of child abuse. This guidance is relevant to any child abuse. NOTE: A Disposal Freeze for government records relating to children is currently in force for Western Australian State and local government. This freeze will remain in place until further notice.

Steps	Additional Information
d) If the alleged perpetrator is a caregiver, it may be necessary to update the Contact Sheet in the Medical Record, and the associates listed in the child's PSOLIS / webPAS record, to ensure that CAMHS staff do not inadvertently contact the alleged perpetrator. e) Update the child's individual PSOLIS Management Plans and alerts as required. f) Keep copies of all correspondence, including mandatory reporting documentation, in the medical record. Brief any relevant staff regarding ongoing risk issues (this may include medical/allied health staff). Consult with a senior clinician if you are not sure which other staff members may need to be informed.	For advice regarding the release of information to external parties, refer to: • CAHS Mediation and Legal Support Service – Release of Information • Guidelines for Protecting Children 2020 (see pg. 61- <i>Sharing information for the protection of children</i>) • CAHS Confidentiality, Disclosure and Transmission of Health Information Policy.

Training

The following CAMHS training programs are relevant to this policy:

- Sexual Safety online training (mandatory for all CAMHS staff)
- Trauma Informed Care training (non-mandatory)
- Mandatory Reporting of Child Sexual Abuse (mandatory for all medical and nursing staff)

CAMHS staff can also access training provided by State-wide Protection of Children Coordination (SPOCC) Unit. Information is available on the [SPOCC intranet page](#).

Related CAHS internal policies, procedures and guidelines
CAHS Child Safety and Protection Policy
CAHS Mandatory Reporting of Child Sexual Abuse
CAMHS Shared Care Guidelines
CAMHS Transitions in Care Policy
CAMHS Sexual Safety Guidelines
Fact sheet MHA 2014 Charter of Mental Health Care Principles

Children and Community Services Act 2004
Children and Community Services Amendment (Reporting Sexual Abuse of Children) Act 2008
CAMHS Multidisciplinary Team Review Guidelines
CAMHS Clinical Assessment Policy
CAMHS Clinical Engagement
CAMHS Clinical Supervision
CAMHS Risk Assessment and Management Policy
State Records Office of Western Australia - Guidance for identifying and retaining records which may become relevant to an actual or alleged incident of child sexual abuse
WA Health Guidelines for Protecting Children 2020
WA Health Responding to an Allegation of Sexual Assault Disclosed within a Public Mental Health Service

References and related external legislation, policies, and guidelines
¹ Kogan S M. 2005. The role of disclosing child sexual abuse on adolescent adjustment and revictimisation.
² Gries L T, et al. 2000. Positive reaction to disclosure and recovery from child sexual abuse.
³ Nagel D E, et al. 1997. Disclosure patterns of sexual abuse and psychological functioning at 1-year follow-up.
⁴ Fergusson D M, et al. 2000. Risk factors and life processes associated with the onset of suicidal behaviour during adolescence and early adulthood.
⁵ Ungar M, et al. 2009. Patterns of disclosure among youth.
⁶ Ullman S E. 2002. Social reactions to child sexual abuse disclosures: A critical review.
⁷ Bolen R M, & Lamb J L. 2004. Ambivalence of non-offending guardians after Child Sexual Abuse Disclosure.
⁸ Goodman-Brown T G, et al. 2003. Why children tell: a model of children's disclosure of sexual abuse.

Useful resources (including related forms)
Department of Communities- Signs of Safety: Child Protection Practice Framework (2nd ed., 2011).
Department of Communities - Rapid Response: Prioritising services for children and young people in Care .
Department of Communities - Mandatory Reporting of Child Sexual Abuse portal
Commissioner for Children and Young People – Child Safe Organisations
Victims Services, NSW Department of Justice: Good practice in providing services: Victims of child abuse
National Institute for Health and Care Excellence – NICE Guideline [NG 76]: Child abuse and neglect

This document can be made available in alternative formats on request.

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*Note: CAMHS Leadership Committee approved a 2-month extension of the review date to 07.06.2024.

Appendix 1: Statement of applicability against the Charter of Mental Health Care Principles in the *Mental Health Act 2014* and Child Safe Standards developed by the Royal Commission into Institutional Responses to Child Sexual Abuse

<u>Charter of Mental Health Care Principle</u>	<u>Child Safe Standard</u>	<u>Applicability to guideline</u>
Principle 1: Attitude towards people experiencing mental illness	-	Abuse violates a child's rights and dignity, and can cause shame, guilt, and stigma. Staff acknowledge the impact that the abuse has had on the child and their family
Principle 2: Human rights	-	Appropriate steps are taken to uphold the right of children and their families to be free from abuse and maltreatment, including its harmful effects.
Principle 3: Person-centred approach	-	CAMHS delivers trauma-informed care which takes into account individual trauma history and other individual factors that may impact on their experience of care. As abuse may not be disclosed until sometime after the child and family enters CAMHS, and may be an ongoing process, all aspects of service delivery, from assessment to discharge and transfer, should be trauma informed.
Principle 4: Delivery of treatment, care and support	-	Staff acknowledge the unique barriers that children who have experienced abuse and their families may encounter when accessing mental health services. Insofar as is practicable, services are delivered in an environment that enhances the child's physical and emotional safety. This includes taking appropriate measures to

		address aspects of the environment that may increase risk of re-traumatisation.
Principle 5: Choice and self-determination	Standard 2: Children participate in decisions affecting them and are taken seriously	Where appropriate, CAMHS staff involve the child and their family in decisions relating to the management of risk following the disclosure. This occurs within the bounds of clinicians' professional obligations and duty of care, and in a way that is appropriate to the child's age, mature minor status, and individual circumstances.
Principle 6: Diversity Principle 7: People of Aboriginal or Torres Strait Islander Descent	Standard 4: Equity is upheld and diverse needs are taken into account	Staff consider culture, gender, age, sexuality, disability, religious and spiritual beliefs and practices, and other factors that may impact on the process of disclosure, reactions to the disclosure or discovery of the abuse, and recovery from the abuse. This may include the use of traditional Aboriginal healing approaches or the involvement of spiritual/religious members (depending on the preferences of the young person).
Principle 8: Co-occurring needs	-	Staff acknowledge that children who have experienced abuse are more likely to have co-occurring needs, including health needs resulting from or related to the abuse, and facilitate access to appropriate health services (including forensic medical services) where required

Principle 10: Privacy and confidentiality Principle 12: Provision of information about mental illness and treatment Principle 13: Provision of information about rights	-	Staff explain confidentiality and privacy (and its limitations), including duty of care and mandatory reporting requirements, and steps that will be taken following the disclosure of abuse, to children and their families in a way that will help them to understand, and express their views and concerns.
Principle 14: Involvement of other people	Standard 3: Families and communities are informed and involved	Children who have experienced abuse, and their families, have a right to comprehensive and integrated services that best supports their physical and emotional safety and meets their individual needs. Where indicated, and in line with duty of care, CAMHS engages other people (e.g. support persons) or facilitates access to other services (e.g. Child Protection and Family Support, Schools) to provide a care plan that is culturally and clinically safe and effective.

Appendix 2: Responsibilities (who to contact)

Organisation/ Position	Description/ aim of the organisation or position	Specific responsibilities related to CSA	CAMHS' relationship with this organisation/ position	Contacts and website
Internal to WA Health:				
Child Protection Consultation Liaison (CPCL) <i>Location:</i> Community CAMHS	As part of Community CAMHS, CPCL workers provide treatment and care to children with complex and persistent mental health problems, and their families.	CPCL clinicians also provide education and training on child protection and mental health issues for Community CAMHS and Department of Communities staff, respectively; coordinate regular meetings to discuss cases; and provide direct clinical services within Community CAMHS. CPCL clinicians also provide consultation to other Community CAMHS clinicians.	CPCL clinicians work within Community CAMHS.	Contacts: A list of current contacts is available by contacting the CAMHS Corporate Office. The CAMHS Finance and Business Analyst will generate a current list of individuals held against CPCL positions.
Child Protection Unit (CPU) <i>Location:</i> Perth Children's Hospital	Provides medical, forensic, social work and therapeutic services for children and families who have experienced abuse, along with training and education of other professionals	<i>Medical services:</i> Provides medical assessment and treatment (including diagnosis and treatment of injuries and infections), conducts medical examinations and reports for forensic evaluations and legal proceedings. <i>Social work services:</i>	Coordinates child protection assessment and intervention (including medical evaluation and care) and provides consultation on child protection issues. Refer to the CAHS Child Safety and Protection policy for details of relationship between CAMHS and CPU.	Contact: <i>During business hours:</i> Duty Social Worker - 6456 4300 <i>After hours/ weekends:</i> call the after-hours Social Worker or CPU on-call Doctor via the Perth Children's Hospital Switchboard.

Organisation/ Position	Description/ aim of the organisation or position	Specific responsibilities related to CSA	CAMHS' relationship with this organisation/ position	Contacts and website
	working with children.	Assessment of ongoing safety of the child and family (from a child protection perspective), psychosocial risk assessment, crisis support and advocacy. <i>Therapy services:</i> Work as a therapeutic team providing trauma-informed individual, family, and group therapy for children and families where child abuse has occurred. Service is free and not time-limited.		Email: PCHcpuduty@health.wa.gov.au Intranet page: https://pch-healthpoint.hdwa.health.wa.gov.au/directory/clinicalservices/Cpu/pages/default.aspx
Sexual Assault Resource Centre (SARC) <i>Location:</i> King Edward Memorial Hospital (KEMH)	Provides medical and psychological care to individuals who have experienced sexual assault or abuse.	Provides services to young people 13 years and up and adults who have experienced recent or past sexual assault, current or past sexual abuse. This includes: Emergency Services, including medical, forensic and counselling support up to two weeks	CAMHS clinicians may refer or give the contact details for the SARC: - Emergency Service to the adolescent and parent(s) if the adolescent has experienced a recent sexual assault (i.e. within the past 14 days). - Crisis Line if the adolescent has experienced past (including recent)	Contacting in an Emergency after a Sexual Assault (Only available if the sexual assault occurred within the past 2 weeks) - You can telephone SARC at any time of the day or night following a recent sexual assault. - Tel: (08) 6458 1828

Organisation/ Position	Description/ aim of the organisation or position	Specific responsibilities related to CSA	CAMHS' relationship with this organisation/ position	Contacts and website
		after a sexual assault. • Counselling services for recent and past sexual assault and sexual abuse. • Advise to health professionals. • Education and training for professionals. Some services are available 24 hours.	sexual assault or abuse. • Counselling Service if the adolescent has experienced sexual assault or abuse in the past;	• Tel: 1800 199 888 (free call from landlines) Contacting for Crisis Counselling • If you are in crisis emotionally, you can speak to a SARC counsellor over the telephone between 8.30am and 11pm any day of the week. • Tel: (08) 6458 1828 Contacting to Request a Counselling appointment • You need to telephone SARC between 8.30am and 4.30pm to request a counselling appointment. • Tel: (08) 6458 1828 General administration queries and appointment changes/cancellations

Organisation/ Position	Description/ aim of the organisation or position	Specific responsibilities related to CSA	CAMHS' relationship with this organisation/ position	Contacts and website
				Tel: (08) 6458 1820 Intranet page: KEMH SARC
Statewide Protection of Children Coordination (SPOCC) Unit <i>Location:</i> Perth Children's Hospital	Delivers a range of initiatives, including policy, consultation, and training, to promote a whole of health system approach to protecting children.	Works with WA Health service providers (including CAHS / CAMHS) to implement mandatory reporting legislation, promote the protection of children and prevention of abuse and neglect, strengthen processes between agencies working to protect children, and develop and evaluate processes for the protection of children in WA Health.	Resources, training Consultation and advice	Contact: SPOCC Unit Phone: (08) 64560030 Email: spoccunit@health.wa.gov.au Intranet page: https://cahs-healthpoint.hdwa.health.wa.gov.au/directory/SPOCC/Pages/default.aspx
External to WA Health:				
Department of Communities including the childFIRST Team	Department of Communities provides a range of child protection and support services to individuals and families across WA.	Department of Communities (including members of the childFIRST Team) work in collaboration with CPU, SARC, WA Police, and CAS to investigate allegations of CSA. The Department of Communities conduct child protection assessments of	As outlined above, CAMHS clinicians report disclosures of/suspected CSA to Department of Communities. The Bilateral Schedule between CAMHS and Department of Communities outlines the responsibilities of each Agency. WA Health is also a	For mandatory reports of child sexual abuse: Contact the Department of Communities Mandatory Report Service. For non-mandatory reports of child sexual abuse: Contact the District

Organisation/ Position	Description/ aim of the organisation or position	Specific responsibilities related to CSA	CAMHS' relationship with this organisation/ position	Contacts and website
		ongoing risk using the Signs of Safety Child Protection Practice Framework to explore harm, danger, safety, and strengths for the child and family. Even if no charges are laid following a police investigation, Department of Communities can still do a child protection assessment and determine if the child is at risk from the accused person, particularly if the alleged perpetrator is in a caregiving role. Apart from having a statutory reporting role to assess and respond to CSA, Department of Communities provides or arrange for the provision of support to PCH and counselling services to the child and family, including child sexual abuse therapeutic services (CSATS).	signatory to the Department of Communities-led Rapid Response. Prioritising services for children and young people in care	Office in the area the child lives in. https://www.dcp.wa.gov.au/Pages/Ho me.aspx For contact information for CSATS services: Go to Services in the Community webpage
Child Abuse Squad (CAS)	As part of the WA Police, CAS aims to	CAS complete forensic interviews following disclosure	Where there are indications in the early stages of the	WA Police number for non-

Managing Clinical Risk After Disclosure of Child Sexual Abuse

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	investigate cases of alleged child sexual and physical abuse. The unit is staffed by WA Police, detectives, and specialist child interviewers, and works jointly with the Department of Communities childFIRST Team.	of child sexual and physical abuse. The interview is video recorded and if charges are laid the recording may be used in court.	investigation that the child may be involved with, or require support from, CAMHS, CAS will encourage supportive contact. CAS may ask for information from Department of Communities, Perth Children's Hospital CPU, and SARC (if they have seen the child) and from CAMHS, if CAMHS is involved.	emergencies: 131 444 Websites: Police WA Your-Safety Child Abuse WA Police How to report child abuse WA Police Your Safety - Sexual Assault
The Office of the Director of Public Prosecutions (ODPP)	The ODPP are responsible for the court proceedings, since CSA and sexual assaults are considered crimes against the State,	Prosecution of CSA and sexual assault cases.	There can be very long delays before a case goes to court (up to a couple of years). CAMHS clinicians may have a role in supporting the mental health needs of the child and their family in the period leading up to, and during, the court case.	Phone: 9425 3600 Email: dpp@dpp.wa.gov.au Website: http://www.dpp.wa.gov.au/
The Child Witness Service (CWS; Department of Justice)	Provides information and support to child witnesses and their families. Children/young people are referred to the Service if/when	Provides preparation and support for children who may be required to testify in court, and their families (if the accused pleads Not Guilty the child will need to give	CWS will undertake to assess whether other Agencies', including CAMHS, involvement with a potential child witness at the time of referral. CAMHS clinicians may liaise with a child's Child Witness	Contact: Victim Support and Child Witness Service Phone: 1800 818 988 Website: Child Witness Service

Organisation/ Position	Description/ aim of the organisation or position	Specific responsibilities related to CSA	CAMHS' relationship with this organisation/ position	Contacts and website
	charges are laid against the alleged perpetrator of CSA.	evidence in a trial). • Undertakes assessments of, and advocate for, a child's needs in relation to giving evidence. • Works with the WA Police and ODPP to ensure the child and family are kept updated on the progress of the case. • Helps potential child witnesses prepare a victim impact statement.	Support Officer (CSWO) to collaboratively assess, advocate for, and support the emotional needs of the child. CWS does not provide counselling services, but can refer to other agencies when required.	